Partnership shortlist

Historical snapshot — October 10, 2026. Older app-status statements and commercial alternatives reflect this assessment date. See current implementation and the selected integrated device, app, and model research MVP. The existing-products/refill service is an alternative, not the selected MVP. Original evidence limits remain.

[Product Name] | United States | October 10, 2026 (America/Chicago)
Branding: the final product name is undecided. [Product Name] is the outreach placeholder; the repository name is an identifier, not a finalized brand.
Goal: a consumer product that helps people judge their own cannabis-related impairment, with a separate future alcohol channel.
Constraint: limited founder funding; prioritize a small feasibility milestone over a complete device-development contract.
PDF: Partnership shortlist with email drafts.
Editable drafts: Partner outreach emails.

Recommendation

Start with Johns Hopkins and UC San Diego for scientific fit, and Healgen for an existing-test supply inquiry. MGH is a targeted lead if custom saliva sensing remains essential; BACtrack is useful later for the separate alcohol channel.

These are prospects to approach, not confirmed partners or investors. Priority reflects technical fit and the proposed first ask, not a measured probability of acceptance. No inquiries were sent. Current partnership interest, pricing, capacity and commercial terms are unverified.

The market split matters: a chemical test can establish exposure, while an impairment product needs an independently defined functional outcome. An assay supplier alone cannot supply the missing impairment validation. A useful first partnership should resolve one of those questions at a bounded cost.

Five priority prospects

Priority	Prospect	Best role	Proposed first ask
1	Johns Hopkins Behavioral Pharmacology Research Unit (BPRU)	Cannabis pharmacology and research-method collaboration	Scope a feasibility review of oral-fluid THC plus task/physiology against an independent functional endpoint. Explore a joint pilot or grant proposal if there is scientific fit.
2	UC San Diego Center for Medicinal Cannabis Research (CMCR)	Controlled cannabis research and impairment-outcome expertise	Discuss endpoint selection and whether an ancillary measurement proposal could fit an approved study, subject to institutional review.
3	Healgen Scientific	Existing oral-fluid THC screening products	Request current instructions, exact orderable OTC THC product, supported user workflow and small-volume US supply terms.
4	Massachusetts General Hospital Center for Systems Biology / EPOCH assay researchers	Targeted saliva-THC sensing or technology collaboration	Ask about the technology's current status, license/collaboration route and a bounded native-saliva feasibility comparison.

Priority	Prospect	Best role	Proposed first ask
5	BACtrack	Separate breath-alcohol products and app integration	Ask about supported models, resale/SDK terms, result-export rights and calibration responsibilities. Defer until the alcohol branch is needed.

Public contact details

Prospect	Contact details / public route	Verification
Johns Hopkins BPRU	Institutional BPRU page. Ask for the cannabis pharmacology research team associated with the cited Vandrey work.	Current email, phone and personnel not verified.
UC San Diego CMCR	CMCR institutional site. Ask for research administration or cannabis impairment investigators.	Current email, phone and personnel not verified.
Healgen Scientific	Manufacturer site. Optional referral: **Jenny Xia, LSI International Inc.; jxia@lsi-consulting.org; 301-525-6856**.	Contact details verified in the FDA sponsor summary, summary page 1. This is a listed regulatory correspondent, not Healgen sales; current representation is unconfirmed.
MGH / EPOCH	Center for Systems Biology or Mass General Brigham Innovation. Ask for the EPOCH investigators or technology licensing/business development team.	Current email, phone, ownership and licensing availability not verified.
BACtrack	Official SDK page. Ask for developer/business integration or wholesale inquiries.	Current email, phone and inquiry channel not verified.

The Healgen referral details come from the public FDA filing. Use them to request the appropriate manufacturer contact, not as a verified sales inbox. Other websites remain blocked in this environment; no email address has been guessed. The email drafts identify which recipient fields need verification before sending. Nothing has been sent.

Why these prospects fit—and what is unconfirmed

1. Johns Hopkins BPRU

The repo cites Vandrey and colleagues' research connecting blood/oral-fluid cannabinoid disposition with pharmacodynamic outcomes. That overlaps the project's central chemistry-plus-function question. The proposed first deliverable is a protocol/feasibility assessment, not development of the finished device. A jointly developed study or funding proposal would require agreement on responsibilities, approvals and costs.

Public route: BPRU institutional page. **Evidence:** 2017 oral-cannabis pharmacokinetic study, already cited as SCI-02 in the repo. Institutional availability and current personnel were not verified live. Research collaboration is not assumed free.

2. UC San Diego CMCR

CMCR is a strong candidate for evaluating independent impairment outcomes. Marcotte and colleagues' driving-performance trial is a relevant literature lead, but driving performance is not automatically equivalent to a general impairment score. Any study extension would require the institution's scientific and regulatory approval.

Public route: CMCR. **Literature lead:** 2022 driving-performance trial. The site and this citation were not independently retrieved in this session; confirm current investigators, study capacity and relevance before relying on the lead. There is no evidence that CMCR would fund this project.

3. Healgen Scientific

Strongest verified existing-test supplier lead. FDA records for K252523 confirm an OTC qualitative oral-fluid cotinine/THC screening system; the THC cutoff is 40 ng/mL. Results are preliminary; confirmatory laboratory testing is a separate step. This supports exposure screening, not impairment. The additional THC lay-user study paired users and explicitly states that no one self-tested. Obtain the actual instructions before assuming the desired solo-use workflow is supported.

Public route: manufacturer. **Verified primary evidence:** FDA decision summary and clearance letter / sponsor summary. Current standalone THC SKU, inventory, price, domestic distribution, private-label and OEM rights remain unverified. A finished-product supply arrangement would deliver a narrower benefit than the intended impairment device.

4. MGH / EPOCH researchers

The repo's chemistry review cites Yu and colleagues' rapid oral-fluid THC assay. It is a targeted lead for the chemical-measurement layer, rather than an established consumer detector or impairment model. Request current technology ownership, commercialization status, evidence access and license terms before assuming the published platform can be used.

Public routes: Center for Systems Biology and Mass General Brigham Innovation. **Evidence:** 2021 EPOCH assay paper, cited as reference 35 in the repo chemistry review. Current technology availability, licensing and collaboration terms were not verified live.

5. BACtrack

The repo's earlier source review documents an iOS/Android SDK for connected breath-alcohol products. This could support a separate alcohol offering; it does not authorize a new impairment score or modified measurement workflow. An alcohol development branch should not hold up the first cannabis-feasibility milestone.

Public route: BACtrack SDK. Current live access was blocked. Confirm models, actual supported functions, read-only result import, rights, minimum volumes and commercial terms. This is a supplier/integration lead, not a verified funding source.

How to approach them with limited money

Prepare one page containing the user, intended result, current prototype status, scientific question and requested first deliverable. Be clear that the repo contains synthetic tools and specifications, with no validated assay or impairment model. Ask for a short fit discussion and a written scope before commissioning work.

For the research prospects, explore a small sponsored feasibility study, a joint funding proposal or relevant existing-study collaboration. These options can still require funding; grant eligibility, open calls and awards have not been verified here. For suppliers, ask for the exact labeling and a small-volume quote before discussing a custom instrument.

Establish data rights, background/foreground IP, publication terms, evidence access, costs and stop/go criteria. No candidate is established as willing to invest, donate work or share proprietary evidence.

Reserve paid-development option: DCN Dx is a provisional lateral-flow/point-of-care development lead if finished-product sourcing cannot fit the project. Its current capabilities and pricing were not verified live. Consider only a bounded feasibility quote once funding is available.

Research status

FDA records for Healgen were retrieved and reviewed. Several institutional and manufacturer sites returned network-policy denials; those entries are explicitly provisional or grounded in the repository's earlier cited research. The necessary website domains were saved in the environment draft, preserving existing settings; that save does not itself enable access or publish the environment.

This shortlist does not verify current contacts, available partnerships, grant awards, supplier prices or a legally marketable impairment product. The next validation is current-page/contact verification and direct expressions of interest; no outreach has been performed.

Repository context: PRD, scientific source register, THC chemistry review, faster market-entry strategy and cost assessment. These recommendations do not change product requirements or close release gates.

Partner outreach email drafts

Historical snapshot — October 10, 2026. Older app-status statements and commercial alternatives reflect this assessment date. See current implementation and the selected integrated device, app, and model research MVP. The existing-products/refill service is an alternative, not the selected MVP. Original evidence limits remain.

[Product Name] | October 10, 2026 (America/Chicago)
Status: unsent drafts. Replace [Product Name], [Your Name], [Your Email] and any other bracketed field before use. The final brand is undecided.
Shortlist and contact evidence: Partnership shortlist. The combined PDF contains the shortlist and these drafts.

Recipient addresses are unverified except the FDA-listed Healgen regulatory correspondent below. Institutional/vendor routes are provided in the shortlist; verify the current recipient before sending. No messages, calls or introductions have been made.

1. Johns Hopkins BPRU

To: [Current BPRU cannabis research contact — verify email]
Subject: [Product Name] — scoped cannabis-assessment feasibility discussion

Hello BPRU team,

I'm developing an early-stage project, [Product Name], intended to help adults assess their own cannabis-related functional impairment. The proposed workflow combines oral-fluid THC measurement, a reaction task and physiological measurements.

Your published work connecting cannabinoid disposition with functional effects is relevant to our question: whether these measurements can predict an independently defined functional outcome. We currently have specifications and synthetic software tools, with no validated assay or impairment model.

With a limited initial budget, I'm seeking a brief fit discussion and a bounded feasibility assessment covering endpoint selection, scientific risks and the next useful milestone. If appropriate, I'd also welcome discussing a joint pilot or funding proposal.

Could you direct me to the appropriate investigator and explain your engagement process and initial costs?

Thank you,
[Your Name]
[Your Email] | [Your Phone, optional]

2. UC San Diego CMCR

To: [Current CMCR research inquiry contact — verify email]
Subject: [Product Name] — cannabis impairment endpoint and feasibility inquiry

Hello CMCR team,

I'm exploring [Product Name], a consumer cannabis-assessment concept combining oral-fluid THC, a reaction task and physiological measurements to help adults assess their own functional state.

We are at the specification and synthetic-prototype stage, without a validated assay or impairment model. I'm seeking advice on an independent functional endpoint and whether the proposed measurements merit further study.

Would your group consider a brief fit discussion followed by a narrowly scoped feasibility review? If scientifically appropriate, I'd also like to understand whether an ancillary measurement proposal might fit an existing approved study, subject to your institutional review and agreements.

Our initial budget is limited. A clear first deliverable, cost estimate and stop/go criteria would help us plan responsibly.

Thank you,
[Your Name]
[Your Email] | [Your Phone, optional]

3A. Healgen contact referral

To: jxia@lsi-consulting.org
Listed contact: Jenny Xia, LSI International Inc.; telephone 301-525-6856.
Role/source: regulatory correspondent in the FDA K252523 sponsor summary, summary page 1. Current representation and referral availability are unconfirmed.
Subject: Referral to Healgen oral-fluid test distribution contact — [Product Name]

Hello Ms. Xia,

I found your contact information in the public FDA summary for Healgen's K252523 submission. I'm developing an early-stage US project, [Product Name], and would appreciate a referral to the appropriate Healgen sales or distribution contact for its oral-fluid THC screening products.

Our budget is limited. We want to explore a small initial purchase of an existing finished product within its current labeling, starting with availability, minimum order quantities and instructions for use.

I understand your listed role is regulatory correspondence. Could you point me to the appropriate business contact?

Thank you,
[Your Name]
[Your Email] | [Your Phone, optional]

3B. Healgen substantive supplier inquiry

To: [Current Healgen sales/distribution contact — verify email]
Subject: Small-volume US distribution inquiry for OTC oral-fluid THC testing — [Product Name]

Hello Healgen team,

I'm developing [Product Name], an early-stage US consumer project. We are evaluating an optional first offering using an existing finished oral-fluid THC screening test, preserving its labeling and pairing it with a simple result-history/refill service.

Could you share the currently available OTC THC-containing products associated with K252523 and their current instructions? We need to understand whether the supported workflow permits solo self-testing or requires another person to collect or interpret the test.

Please also advise on sample availability, the smallest initial order, US wholesale pricing, lead time, shelf life, domestic distribution options and arrangements for confirming preliminary positive results.

Our initial budget is limited, so we'd prefer a small supported arrangement before considering custom hardware or private labeling. We understand that preliminary THC screening does not establish impairment.

Thank you,
[Your Name]
[Your Email] | [Your Phone, optional]

4. MGH / EPOCH technology inquiry

To: [Current EPOCH research or MGB licensing contact — verify email]
Subject: [Product Name] — EPOCH saliva-THC technology and feasibility inquiry

Hello Center for Systems Biology / Innovation team,

I'm developing an early-stage cannabis-assessment concept, [Product Name], and read the 2021 EPOCH paper on rapid oral-fluid THC measurement.

We have specifications and synthetic software tools, with no validated saliva assay or impairment model. We are evaluating the chemical-measurement component and understand that detecting THC does not establish impairment.

Could you share the technology's current commercialization status, appropriate contact and any available licensing or research-collaboration route? With a limited initial budget, we'd prefer a bounded feasibility discussion about native-saliva performance, reference-method comparison and expected development requirements.

If a suitable engagement is possible, please let us know the process, first deliverable and approximate cost.

Thank you,
[Your Name]
[Your Email] | [Your Phone, optional]

5. BACtrack developer / business inquiry

To: [Current BACtrack developer/business contact — verify email]
Subject: Small-volume SDK and distribution inquiry — [Product Name]

Hello BACtrack team,

I'm developing an early-stage US project, [Product Name]. We are exploring a separate future alcohol offering using an existing BACtrack device and a simple result-history service.

Our budget is limited. Could you advise on the smallest supported arrangement for evaluating a device and developer integration?

We'd like to understand supported models, SDK access/licensing costs, small-volume resale terms, calibration/support responsibilities and whether an app can receive and faithfully display results without initiating or changing the measurement workflow.

Our initial scope would preserve the device's supported use and labeling. We would not derive a combined THC/alcohol impairment score from these results.

Please point me to the appropriate developer or business contact and current documentation.

Thank you,
[Your Name]
[Your Email] | [Your Phone, optional]